BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Routine blood safety checks are different from tests ordered for a symptom and from scans used to assess the cancer. Your team supplies exact dates and explains results.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ tests	Before starting: full blood count, kidney and liver tests, plus measured kidney filtration (preferred) or an appropriate estimate for carboplatin dosing. Review prior treatment, medicines, hearing and nerve symptoms. Arrange dental, nutrition and swallowing preparation for head and neck radiation. Audiology and formal nerve assessment are arranged when clinically indicated.	Full blood count and kidney and liver tests are required before every weekly cycle. Kidney results guide the carboplatin dose. Review infection or bleeding symptoms, mouth and skin pain, weight, intake and swallowing throughout radiation.	Report symptoms promptly; extra blood or organ tests may be needed.	The team reviews recovery, results and earlier side effects before another dose or phase.	Mouth, throat and skin symptoms can continue after the last session. The team follows blood-count recovery, nutrition, swallowing and any kidney, hearing or nerve problems. Keep dental care and the agreed radiation late-effect follow-up. Scans, examinations and any thyroid or other tests follow the individual treatment plan; there is no universal scan or tumour-marker timetable here.
Treatment and appointments	Confirm the exact regimen, treatment aim, prescription and appointment calendar.	Carboplatin AUC 2 is infused through a vein over 30 minutes. The dose in milligrams is calculated by the treating team using kidney function. Allow extra time for assessment, preparation and prescribed anti-sickness treatment, as well as separate radiation appointments. This weekly carboplatin regimen does not automatically require the cisplatin hydration or docetaxel steroid schedule from an earlier phase.	Follow the written support plan and record symptoms and medicines taken.	After chemoradiation, the team reviews recovery, response and whether any further treatment is appropriate. National Cancer Control Programme (NCCP) 00419 refers to later carboplatin/fluorouracil under 00552, but 00552 is specific to nasopharyngeal cancer. It is not automatically applicable to every head and neck cancer; any further chemotherapy requires a separately confirmed indication, prescription and start date.	Record the final treatment dates and the named follow-up team.
Mouth, nutrition and swallowing	Agree oral care and assess weight, intake and swallowing. Arrange dental review before planned head and neck radiation.	Review mouth pain, weight, drinking, swallowing and, during radiation, the treated skin.	Report poor intake, choking, weight loss or inability to drink.	Dietitian and speech and language plans adapt to symptoms and recovery.	Continue dental, nutrition and swallowing care as needed.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Tumour assessment and imaging	Confirm the cancer site, stage and tests relevant to that site.	Discuss symptoms and the agreed treatment aim.	Examinations or scans may be arranged for a particular concern.	Response, recovery and your preferences inform the next treatment decision.	Use your individual scan and clinic plan; this regimen does not set one universal scan or blood tumour-marker schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00419 v6: official regimen · NCI: head and neck cancers · NCI: oral complications of cancer therapies, patient version · NCI: nutrition during cancer treatment

Tests in your plan

Full blood count

WHEN

At baseline and before every weekly carboplatin cycle.

Checks infection-fighting white cells, red cells and platelets; low counts may require a delay or another change.

Kidney function and filtration assessment

WHEN

Before starting and before each weekly cycle. The team determines the required recency of creatinine results for the calculation method.

Carboplatin dosing depends on kidney function. Measured GFR is preferred where feasible, with an appropriate estimate used when needed.

Liver function

WHEN

At baseline and before every weekly cycle.

Supports safe treatment review and investigation of changes.

Hearing and nerve review

If part of your plan

WHEN

Review symptoms before dosing; hearing tests or further examination when indicated.

Identifies new problems and effects remaining from earlier platinum chemotherapy.

Dental, nutrition and swallowing assessment

WHEN

Prepare before head and neck radiation; reassess as symptoms or intake change.

Helps plan safe intake, mouth care and prevention of avoidable radiation-related complications.

Imaging and cancer assessment

If part of your plan

WHEN

Examinations and scans on the individual oncology and radiation plan.

Assesses the cancer and informs further treatment.

THE WHOLE TREATMENT JOURNEY

Carboplatin AUC 2 · weekly head and neck chemoradiation: treatment and checks

NCCP 00419a: stage III/intravenous (IV) locally advanced head and neck squamous cell carcinoma when cisplatin is contraindicated or not tolerated. This authored content is restricted to the head_neck context; cervical content remains separate.

Carboplatin AUC 2 IV every 7 days for 6 cycles with radiotherapy. This is the current NCCP 00419 v6 head and neck schedule; do not substitute the AUC 1.5 seven-cycle post-induction pathway.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment phase NCCP	Confirm the full multidisciplinary team (MDT) plan and individual treatment calendar	Carboplatin AUC 2 IV every 7 days for 6 cycles with radiotherapy. This is the current NCCP 00419 v6 head and neck schedule; do not substitute the AUC 1.5 seven-cycle post-induction pathway.	Attend planned radiotherapy and report problems early	Coordinate each weekly treatment with the radiotherapy and recovery plan. No automatic induction, adjuvant fluorouracil or maintenance phase is added by this regimen.	Record the completed medicines and next follow-up plan

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, renal and liver profile; clinical status, medicines, allergy and previous treatment review.	Full blood count, renal and liver profile before each cycle; use the current regimen's exact count and organ-function rules.	Report infection, bleeding, reduced urine or dehydration	Repeat or reassess tests if toxicity or recovery changes the plan	Further blood tests and imaging are arranged by the treating team
Support medicines NCCP	Confirm the moderate-risk antiemetic prescription and rescue plan	No routine docetaxel premedication or cisplatin hydration belongs to this phase	No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or extra supportive-care prescription is specified by NCCP for this phase. Review marrow recovery and infection history individually.	Reconcile all doses, steroid totals and start/stop dates when the phase changes	Review ongoing medicine need with the team
Mouth, nutrition and swallowing NCCP	Assess mouth, teeth, weight and swallowing needs	Review pain, mouth symptoms, oral intake and hydration	Report inability to drink, severe soreness or weight loss	Dietetic, swallowing and dental support according to need	Continue rehabilitation and late-effect care where needed
Urgent contact NCCP	Save the unit's 24-hour contact and fever instructions	Tell staff about new symptoms or reactions	Seek urgent advice for infection, bleeding, chest pain, severe diarrhoea/vomiting or inability to drink	Do not wait for the next routine appointment if acutely unwell	Keep follow-up and urgent-contact instructions available

What each check is for

1 Concurrent chemoradiation

Follow your agreed treatment calendar

Use carboplatin with radiotherapy when the team judges cisplatin unsuitable or not tolerated. The intended benefit and uncertainty must be discussed in the context of the patient's disease, fitness and radiotherapy plan.

Ask / act: Confirm active medicines, written supportive-care plan and review criteria.

Sources: NCCP

2 Next-phase / completion review

After the planned course and recovery review

The team checks response and fitness before the next treatment or follow-up stage.

Ask / act: Ask the team to confirm the next phase and actual dates.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.48